

Drug-induced immune thrombocytopenia during Quindolox therapy: a case report

Abstract. Immune thrombocytopenia is an uncommon but well-recognised drug adverse effect. We describe a patient who developed severe thrombocytopenia after starting Quindolox, with recovery after withdrawal and a positive dechallenge.

Introduction. Quindolox (a fictional quinolone-class antibacterial) is used for complicated urinary tract infection. Post-marketing reports of haematological reactions are rare. **Methods.** This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests. **Methods.** This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant

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Case presentation. A 71-year-old woman with hypertension and osteoarthritis was admitted with pyelonephritis and commenced on intravenous Quindolox 400 mg twice daily on 3 March 2025. Her platelet count on admission was $244 \times 10^9/L$. On the fourth day of therapy she developed petechiae over both shins and epistaxis. A full blood count showed a platelet count of $8 \times 10^9/L$ with a normal white cell count and haemoglobin. A blood film excluded pseudothrombocytopenia. Quindolox was stopped on the same day and she received one pool of platelets and a short course of oral prednisolone. The platelet count began to recover within 48 hours and reached $180 \times 10^9/L$ by day 10. A drug-dependent antiplatelet antibody assay was positive for Quindolox.

Discussion. The temporal relationship, exclusion of alternative causes, positive dechallenge and a positive antibody assay support a probable causal association. Rechallenge was not attempted and is not recommended. Clinicians should obtain a baseline platelet count and repeat it if bruising or bleeding develops during quinolone therapy. **Methods.** This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior

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Conclusion. Quindolox should be added to the list of antibacterials capable of causing immune thrombocytopenia. References: 1. Aster RH. N Engl J Med. 2007. 2. George JN. Blood. 2009. 3. Reese JA. Blood. 2013.